BASELINE → TREATMENT → FOLLOW-UP

Treatment safety, neuroendocrine tumour (NET) response and follow-up

Your team books and reviews each test. Response scans and hormone or tumour markers follow an individual plan; they are not automatically required at every medicine review.

Check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver	full blood count (FBC), renal/liver tests, blood urea nitrogen (BUN) and urine protein	FBC, kidneys and liver every 4 weeks	Earlier assessment if unwell, dehydrated or less urine	Review abnormalities before continuing	Follow unresolved effects
Glucose / blood fats	Fasting glucose, cholesterol and triglycerides	Lipids regularly; fasting glucose as clinically indicated	Report thirst, frequent urine or marked fatigue	Review treatment for diabetes or lipid changes	Individual follow-up
Infection / mouth / lungs	Review active infection; hepatitis B surface antigen and core antibody	Ask about ulcers, cough, breathlessness, infection and rash	New breathing or infection symptoms need prompt contact	Assess lung inflammation versus infection	Follow symptoms after a hold or stopping
NET response / hormone plan	Document tumour site, grade, symptoms and baseline assessment	Scans / markers only as individually scheduled	Track symptoms and separate hormone medicines	Review benefit, burden and personal goals	Agree next treatment and follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00320 v6 · current regimen · EMA product information and patient leaflet

Tests in your plan

5-hydroxyindoleacetic acid (5-HIAA)

If part of your plan

WHEN

Only if requested for your hormone syndrome; the team sets the dates.

Measures a breakdown product of serotonin to help assess selected neuroendocrine tumours.

Often a timed urine collection. Follow the laboratory’s food and medicine instructions.

Chromogranin A

If part of your plan

WHEN

Only when the team considers it informative for your tumour.

A selected tumour-marker result, interpreted with symptoms and scans.

Acid-suppressing medicines and other conditions can alter the result. It is not a routine test before every treatment; do not stop medicines yourself.

Selected pancreatic hormones

If part of your plan

WHEN

Only for the hormone question identified by your specialist.

Examples include gastrin, insulin or glucagon when the diagnosis or symptoms call for them.

The team names the test, preparation and review date; these are not a standard panel for every patient.

THE WHOLE TREATMENT JOURNEY

Everolimus · neuroendocrine tumours: treatment and checks

Progressive unresectable or metastatic pancreatic NET, or well-differentiated G1/G2 nonfunctional gastrointestinal NET under NCCP 00320b/c.

Take the prescribed everolimus dose once daily every day, at the same time and consistently with or without food. There is no planned tablet-free week.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver NCCP 00320	FBC, renal/liver tests, BUN and urine protein	FBC, kidneys and liver every 4 weeks	Earlier assessment if unwell, dehydrated or less urine	Review abnormalities before continuing	Follow unresolved effects

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Glucose / blood fats NCCP 00320	Fasting glucose, cholesterol and triglycerides	Lipids regularly; fasting glucose as clinically indicated	Report thirst, frequent urine or marked fatigue	Review treatment for diabetes or lipid changes	Individual follow-up
Infection / mouth / lungs NCCP 00320	Review active infection; hepatitis B surface antigen and core antibody	Ask about ulcers, cough, breathlessness, infection and rash	New breathing or infection symptoms need prompt contact	Assess lung inflammation versus infection	Follow symptoms after a hold or stopping
NET response / hormone plan NCCP 00320	Document tumour site, grade, symptoms and baseline assessment	Scans / markers only as individually scheduled	Track symptoms and separate hormone medicines	Review benefit, burden and personal goals	Agree next treatment and follow-up

What each check is for

1 Daily everolimus

Continuous

Everolimus is a daily targeted tablet used to help control particular advanced neuroendocrine tumours (NETs). It aims to slow growth; the team balances disease control, side effects and your priorities. This NET guide does not cover the separate kidney-cancer indication.

Ask / act: Confirm the current prescription, tests and support medicines.

Sources: NCCP 00320

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.37